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Lost in the Margins: Understanding Mental Health Disparities and Access Barriers for Roma in Europe

Vedant Vikramaditya

Abstract

This paper examines the significant disparities in mental health outcomes among Romani populations across Europe, a topic that remains under-researched with much of the existing data being country-specific or dated. The central research question driving this analysis is: To what extent do systemic, socioeconomic, and cultural factors drive the disparities in mental health outcomes among Romani populations across Europe? The paper argues that these disparities are a direct result of the complex interplay between systemic antigypsyism, profound socioeconomic marginalization, and entrenched cultural and communication barriers. To address this, a systematic review of secondary sources, including academic articles and reports mostly from 2015 onward, was conducted, followed by a thematic analysis of the findings. The analysis reveals that institutional discrimination within healthcare systems acts as a direct human rights violation, causing significant psychological stress. This is compounded by high rates of poverty, unemployment, and segregated living conditions, which are shown to be critical risk factors for severe mental health issues. Furthermore, the analysis highlights that cultural stigma, a deep-seated mistrust of institutions, and language barriers consistently prevent Romani individuals from seeking and receiving appropriate care. The paper concludes with a discussion of the limitations in current research and proposes policy implications, advocating for culturally sensitive, community-based interventions and a human-rights-based approach to address the root causes of these health inequities.

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Introduction

Mental health is a vital part of our overall well-being, shaping how we think, feel, and act every day. It affects how we handle stress, connect with others, and contribute to our communities. While mental health is important for everyone, access to help and positive outcomes are often unequally distributed across social groups. This is especially true for Romani populations, who are the largest ethnic minority in Europe. Romani communities, including groups like Roma, Sinti, Kale, and Travellers, have a long and rich history across the continent. However, they have consistently faced harsh treatment, discrimination, and social exclusion. The effects of these ongoing challenges on their health, particularly their mental well-being, are a very important area to study.

The history of the Romani people in Europe is marked by centuries of constant persecution and policies that have attempted to force their cultural assimilation. This long history has led to their current difficult living situations, with widespread poverty, limited schooling, poor housing, and high unemployment. These problems create a never-ending cycle of disadvantage that deeply affects all parts of their lives, including their physical and mental health. Beyond these economic and social struggles, Romani communities often experience direct and indirect forms of racism (antigypsyism) and unfair treatment from official groups in many areas, such as healthcare, education, and jobs. These systemic barriers are often made worse by cultural differences and communication problems, which can make it even harder for Romani individuals to get the help they need. This paper aims to understand these complex issues more deeply by addressing the central question: To what extent do systemic, socioeconomic, and cultural factors drive the disparities in mental health outcomes among Romani populations across Europe?

The disparities in mental health outcomes among Romani populations across Europe are driven by the interconnected forces of systemic antigypsyism and institutional discrimination, socioeconomic marginalization, and complex cultural and communication barriers, which collectively perpetuate a cycle of poor mental well-being and severely impede access to appropriate care.

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Literature Review

Romani populations often face deep social exclusion and have widespread health disadvantages. This deep discrimination helps us understand their health problems. Across Europe, Roma people consistently find it very hard to get essential healthcare due to unfair practices within health systems and a clear lack of culturally sensitive care (Alexiadou, 2018). These barriers, combined with high poverty and social exclusion, are human rights violations (Alexiadou, 2018). This means that their unequal access to healthcare is not just a social issue, but a fundamental breach of their rights to health and dignity. When systems fail to provide equitable access, they perpetuate cycles of poor health outcomes. This systemic neglect can lead to Romani individuals suffering from preventable illnesses or having existing conditions worsen due to lack of timely intervention or adequate care. Studies show Roma populations have poorer health than non-Roma, linked to social factors like bad living conditions and unemployment (Medas, 2015). For instance, living in overcrowded or unsanitary housing can lead to higher rates of infectious diseases, and limited job opportunities can mean no health insurance or money for medical needs. These social factors create a constant state of vulnerability, making it difficult for Romani people to maintain good health. This research points to health inequality rooted in long-standing social and economic disadvantages, highlighting a clear pattern of severe health disparities that persist across generations.

Accessing and utilizing healthcare services remains a major challenge for Romani communities. These challenges are complex, stemming from system-wide issues and personal interactions. Roma populations face many barriers to healthcare, including problems with health system organization, widespread discrimination from staff, cultural differences, language issues, and low health literacy (McFadden et al., 2018). Health system organization often includes rigid appointment systems, lack of flexible services, and poor outreach to mobile or geographically isolated communities. This means that services are not set up to meet the unique needs of Romani communities, making access difficult even when they are available. Discrimination from staff can manifest as dismissive attitudes, stereotyping, or even outright refusal of care, creating a deep mistrust in the healthcare system. This mistrust, built on past negative experiences, often prevents Romani individuals from seeking help even when they desperately need it. These barriers create a cycle where Romani people avoid care, worsening their health. The health insurance system in countries like Macedonia also presents significant legal and practical obstacles for Romani people seeking care (Zoon, 2001). Many Romani individuals struggle to obtain necessary documentation or meet residency requirements, which are often prerequisites for health insurance. These bureaucratic

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hurdles and staff biases lead to systemic denial of care, meaning that even when services exist, Romani individuals may be unable to access them due to administrative complexities or prejudice. This indicates the problem lies in deep-seated biases and failures within the healthcare system, leading to delayed or absent care, and often forcing reliance on emergency services, which are not designed for ongoing health management. Such reliance on emergency care is less effective for chronic conditions and mental health issues, further worsening overall health outcomes.

Mental health issues are widespread and severe within Romani communities, demanding special attention. A thorough review confirms common mental health problems like anxiety, depression, and drug abuse are very common among Roma in Europe (Guerrero, Civišová, & Winkler, 2023). This review highlights a need for more research directly involving Romani communities to create suitable guidelines that are culturally appropriate and effective. Current research often lacks the direct input of Romani voices, leading to interventions that may not resonate with their cultural contexts or lived experiences. Specific studies show worrying numbers: suicide attempt rates among Roma in Hungary are much higher than in the non-Roma population (Tóth et al., 2017). This stark difference points to extreme psychological distress within the community. Romani ethnicity is a strong predictor of multiple suicide attempts, especially when combined with hopelessness and long-term unemployment (Tóth et al., 2017). These factors create a compounding effect, where social and economic hardship directly impacts mental well-being to a dangerous degree. These findings point to a deep mental health burden on Romani individuals and communities, underscoring the urgent need for targeted mental health interventions and support systems that address the unique pressures faced by these populations. The consistent evidence across various studies emphasizes that mental health is not merely an individual issue for Romani people. It is a widespread crisis shaped by their unique social and historical circumstances, requiring a comprehensive and community-led approach.

The impacts on children's health are often passed down through generations and worsened by system-wide problems, showing the long-term effects of Romani marginalization. Early mental health problems in children from marginalized Roma communities are strongly linked to their mothers' stress and harsh discipline at home (Filakovska Bobakova, Chovan, & Van Laer, 2024). This suggests a distressing pattern where the difficulties faced by one generation, especially mothers dealing with extreme hardship, can lead to emotional problems and coping styles that are passed down to their children. This intergenerational transmission of trauma and stress creates a cycle that is hard to break without external support. Beyond family life, children's exposure to racism experienced by others (vicarious racism) is also linked to negative emotional and mental health outcomes. Seeing discrimination

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faced by their family can make them feel the world is not fair, causing helplessness and sadness, which affects their mental and physical health (Heard-Garris et al., 2018). This secondhand experience of racism can erode a child's sense of safety and belonging, contributing to long-term psychological distress and a diminished outlook on their future. Also, the quality of education for marginalized Roma children is a basic human rights issue. Chireac and Arbona (2016) highlight that this lack of fair education not only goes against their basic rights but also greatly limits their chances for social and economic progress. Poor schooling deeply affects their overall well-being, future chances, and long-term health, trapping them in a cycle where educational exclusion directly leads to poorer health later in life. This includes reduced opportunities for stable employment and greater exposure to environments that are harmful to mental health. Problems for Romani children are deeply rooted in widespread and unfair system-wide disadvantages, needing full and ongoing efforts to break this cycle of harm and ensure a healthier future.

Past research has clearly shown the health problems faced by Romani people. This includes how common mental health issues are and what causes them. However, there is still a need to connect these problems more directly to the deep roots of unfair treatment, widespread poverty, and communication difficulties. This paper will uniquely bring together these different causes. It will offer a full picture of why mental health remains poor and why getting help is so hard for Romani communities. By examining how these factors work together, this study aims to give a clearer, more useful understanding for creating better policies and support programs. It will move past simply seeing each problem on its own.

Methodology

To study mental health differences among Romani populations, a systematic approach was used to find and analyze important academic writings. Reliable sources were found by searching major academic databases like Google Scholar and JSTOR. The search used keywords such as "Roma mental health," "Roma healthcare access," and "antigypsyism mental health," focusing on articles from mostly 2015 onwards to ensure current findings. The study collected mostly secondary sources, including journal articles, reviews, and reports from government and non-profit groups. These sources gave detailed analysis and summaries of existing research. Academic papers on public health, sociology, human rights, and psychology related to Romani communities were included. Public data from organizations like the European Union Agency for Fundamental Rights (FRA) also provided factual context.

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Evidence was analyzed using a descriptive approach. This involved carefully reviewing each source to find key findings, common mental health issues, the impact of social factors, and barriers to healthcare. The analysis also looked for effective and culturally sensitive ways to help, giving a full overview of current research. This systematic process helped identify repeated patterns across different studies and regions. To structure the analysis and support the main argument, a thematic approach will be used. Findings are grouped into three main themes, matching the thesis: 1) Systemic Antigypsyism and Institutional Discrimination, 2) Socioeconomic Marginalization, and 3) Cultural and Communication Barriers. Each theme will explore how these specific factors affect mental well-being and access to care. This structure will show how these drivers interact and create a complex cycle of disadvantage for Romani populations, clearly presenting how these forces lead to mental health differences in Europe.

Research Findings and Analysis

A. Systemic Antigypsyism and Institutional Discrimination

Discriminatory practices within healthcare systems directly impede access and quality of care, leading to adverse mental health outcomes. Romani people systematically encounter many difficulties in healthcare settings. These are often seen, from a human rights perspective, as abuses that cause poor mental health outcomes (Alexiadou, 2018). For example, Romani women in the Balkans face significant psychological stress when trying to get essential maternal healthcare. This stress comes from personal, internalized, and institutional racism (Janevic et al., 2011). This means that not only do individuals hold prejudiced views, but the healthcare systems themselves have ingrained biases that make it harder for Romani women to receive respectful and timely care during a critical period of their lives. Furthermore, the failure of European Union frameworks meant to help Roma integration to properly deal with antigypsyism contributes to mental health inequalities continuing (Rostas, 2019). These frameworks often do not go far enough to challenge the deep-seated prejudice and discrimination against Romani people, which means that the underlying causes of poor mental health are not fully addressed. This lack of effective policy enforcement allows discriminatory practices to persist, creating ongoing barriers to mental health support and perpetuating a cycle of disadvantage.

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Exposure to discrimination, including experiences where one sees racism directed at others, greatly increases psychological distress and the risk of mental health disorders. Self-reported discrimination has been linked to various psychological disorders, such as psychosis, paranoia, and eating disorders, which are classified in the DSM-5 (a manual used by mental health professionals to diagnose mental disorders). Individuals who frequently face discrimination are about 25% more likely to be diagnosed with a mental health disorder (Reljić, M., 2024). This shows a clear and significant link between experiencing discrimination and developing serious mental health problems. Chronic racism and antigypsyism are identified as major causes of ongoing health differences, including bad mental health outcomes. This is because they create a constant stress response in the body (Reljić, M., 2024). This continuous stress can wear down a person's mental and physical resilience, making them more vulnerable to illness. Beyond direct experiences, vicarious racism, where a child sees racism happening to others, changes their sense of the world as fair and safe. This can lead to feelings of helplessness and despair, which have serious effects on their mental and physical health (Heard-Garris et al., 2018). For instance, a child witnessing their parent being discriminated against by a shopkeeper or a doctor can internalize that experience, leading to anxiety and a sense of insecurity. Research further illustrates this connection.

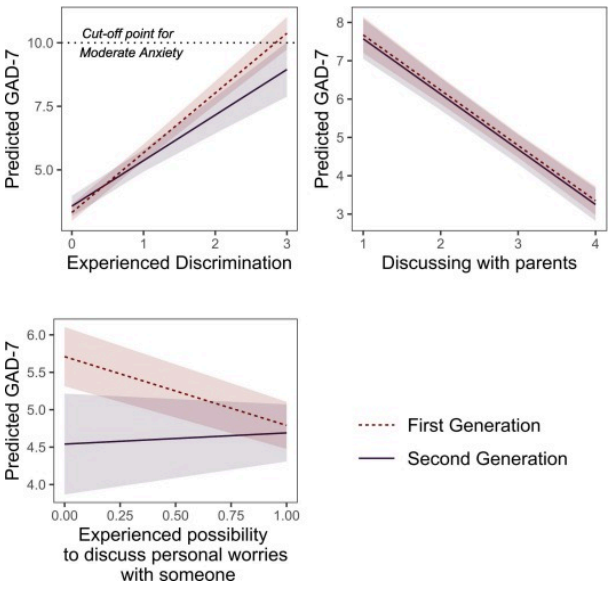


Figure 1: Graph illustrating the correlation between reported experiences of discrimination and prevalence of anxiety/depression

A graph, Figure 1 (Abdulhamed et al., 2022), shows a clear correlation between reported experiences of discrimination and the prevalence of anxiety and depression. This figure typically uses data points to visually represent how as the

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frequency or intensity of reported discrimination increases, so too do the rates of anxiety and depression within the surveyed population, providing strong empirical support for the link between discrimination and mental health outcomes.

B. Socioeconomic Marginalization

High levels of poverty and social exclusion are directly linked to a greater risk for mental health problems. Roma people are exposed to high levels of poverty and social exclusion, which are stated to negatively affect their mental health and well-being (Alexiadou, 2018). This means that the struggle to meet basic needs, such as food, shelter, and safety, creates a constant state of stress that impacts mental health. Research from Slovakia in 2020 indicates that many excluded Roma households experienced long-term stress due to various severe hardships. These included debt (37.6% of households), serious illness (24.8%), cold houses (23.9%), hunger (13.5%), discrimination (12.9%), and forced evictions (10.1%). All these factors severely impact both mental and physical health (Psenkova, A., 2025). This detailed data shows the direct and measurable ways in which socioeconomic deprivation translates into deep psychological distress. The social factors that determine mental health, including poverty and living conditions, are very common among the Roma in Hungary (Medas, 2015). For instance, unstable housing or lack of access to clean water and sanitation contribute to chronic stress and a sense of insecurity, which are known drivers of mental health issues. These findings highlight that the mental health crisis in Romani communities is not merely a consequence of individual vulnerabilities, but a direct result of systemic socioeconomic deprivation.

Segregated living conditions are also important factors in poor mental health outcomes. Living in segregated Roma settlements significantly increases the risk of unfavorable mental health outcomes for Hungarian adults. In fact, individuals living in these settlements face more than twice as high a risk for mental illness compared to the general population (Oláh, B. et al., 2023). This suggests that the conditions of segregation, such as isolation, lack of services, and poor infrastructure, are more impactful on mental health than simply being Romani. Financial status is identified as a key social factor that determines mental health. It directly links economic hardship to poor well-being within these segregated communities (Oláh, B. et al., 2023). A lack of stable income or consistent employment within these settlements means that residents face ongoing financial stress, which is a significant contributor to mental health problems. A visual representation, Figure 2, would typically show a

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chart comparing mental illness rates in segregated Roma settlements versus the general population. This chart would likely illustrate a stark difference, with the bars or lines representing segregated communities being significantly higher, thus emphasizing the profound impact of living conditions on mental health.

Socioeconomic factors like unemployment are critical risk factors for severe mental health conditions, including suicide. Long-term unemployment is identified as a significant risk factor for multiple suicide attempts among Roma in Hungary. This directly links ongoing job insecurity and financial instability to severe mental health outcomes (Tóth et al., 2017). The lack of stable work not only leads to financial strain but also erodes self-worth and hope, which are strong predictors of suicidal behavior. The employment rate for Roma aged 20–64 across surveyed EU countries is approximately 43%, which is much lower than the EU average employment rate of 72% for 2020 (European Union Agency for Fundamental Rights, 2022). This significant difference in employment rates highlights the widespread economic marginalization faced by Romani people. This economic exclusion contributes to a cycle of poverty that directly impacts mental health. Attempted suicide rates among the Roma population in Hungary are 2-3 times higher than in the non-Roma population, showing the severity of mental health challenges within this marginalized group (Tóth et al., 2017). This alarming statistic underscores the urgent need for interventions that address both the socioeconomic roots of distress and provide direct mental health support.

C. Cultural and Communication Barriers

Language and information barriers create substantial obstacles to accessing mental healthcare services. Significant barriers to mental healthcare for Roma are identified, related to common language differences and a severe lack of adequate information regarding available services (Guerrero, Civišová, & Winkler, 2023). This means that Romani individuals may not understand what services are available, how to access them, or even what their mental health condition means, simply because the information is not provided in a language they understand or in a culturally appropriate format. Cultural differences and low health literacy are clearly identified as barriers preventing Roma and Traveller populations from getting and engaging with health services (McFadden et al., 2018). For example, a healthcare provider might use medical terms that are unfamiliar or explain concepts in a way that does not align with traditional Romani understandings of health and illness. This gap in understanding can lead to distrust and a reluctance to engage with formal healthcare systems, even when they are critically needed.

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Cultural stigma and mistrust within Romani communities hinder open discussion of mental health and engagement with formal services. "There is a cultural stigma in many of the Gypsy and Traveller communities around mental health issues, causing people to hide problems from their family and friends, resulting in a delay of asking for help" (Tackling Mental Health Inequalities for Gypsy, Roma, and Traveller People, 2024). This internal stigma means that individuals may suffer in silence, fearing judgment or misunderstanding from their own community if they reveal mental health struggles. Even when Roma do use mental health services, they are often afraid to disclose full details of their condition to professionals. This fear stems from concerns about institutionalization or discrimination, which can impede effective treatment as healthcare providers do not have a complete picture of their needs (Tackling Mental Health Inequalities for Gypsy, Roma, and Traveller People, 2024). This lack of trust is often a result of historical and ongoing negative experiences with state institutions. However, ancestral land connections and cultural identity play a psychosocial role in resilience within Romani communities. This suggests that care that does not respect or incorporate these cultural strengths may fail to truly help (Ivanenko, A., 2024). This highlights the importance of understanding and valuing Romani cultural frameworks when designing mental health interventions.

Community-based and culturally sensitive approaches are essential for effective mental health support. Community-based protective factors, such as supportive environments and spiritual activities promoted by groups like the Iglesia Evangélica Filadelfia, are crucial for developing culturally sensitive mental health support strategies (López, J et al., 2018). These examples show that when support comes from within the community, it is more likely to be trusted and effective because it aligns with existing social structures and beliefs. The need for more participatory research to develop culturally appropriate guidelines for mental health support implicitly acknowledges existing cultural and communication gaps that standard approaches fail to address (Guerrero, Civišová, & Winkler, 2023). This means that Romani communities should be actively involved in designing and delivering mental health services, ensuring that interventions are relevant, accessible, and respectful of their unique cultural contexts. Such approaches can help build trust, reduce stigma, and ultimately improve mental health outcomes by leveraging community strengths and addressing specific needs.

Conclusion

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In conclusion, this paper has demonstrated that the disparities in mental health outcomes among Romani populations in Europe are not the result of a single factor but are instead driven by the complex interaction of systemic antigypsyism and institutional discrimination, socioeconomic marginalization, and significant cultural and communication barriers. The research findings show that Romani people face discriminatory practices in healthcare systems, which are a direct human rights violation leading to stress and psychological distress. This is worsened by widespread poverty, unemployment, and segregated living conditions, which are directly correlated with an increased risk for severe mental health conditions, including suicide. Finally, cultural stigma, historical mistrust of institutions, and language barriers act as a powerful deterrent to seeking and receiving effective mental health care.

A key limitation of the current body of literature is that the field is not extensively researched. Much of the available data is either from single-country studies or is older, making it difficult to draw broad conclusions and compare outcomes across Europe. This gap highlights a critical need for more robust, cross-national, and up-to-date research. Future studies should focus on participatory research models that actively involve Romani communities in data collection and analysis to ensure cultural appropriateness and relevance.

Based on these findings, policy implications are clear and require multiple aspects. Governments and health organizations must move beyond generic integration strategies and develop targeted policies that address systemic discrimination in healthcare settings. This includes implementing mandatory cultural sensitivity training for all staff and providing services in a culturally and linguistically appropriate manner. Furthermore, policies aimed at improving employment opportunities and living conditions in segregated communities are essential to address the root socioeconomic causes of mental health distress. The ultimate significance of this research is in its call for a more holistic and human-rights-based approach to mental health. By understanding these interconnected drivers, we can create more equitable, effective, and sustainable solutions that truly support the mental well-being of Europe's largest ethnic minority.

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